

HOW TO OPEN A HOME HEALTH AGENCY in New Mexico

A licensed, Medicare-certified home health agency — Medicare, Medicaid & private pay.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The New Mexico model — read this first

A home health agency provides SKILLED, medical care in people's homes — nursing, physical/occupational/speech therapy, and home health aide services under a physician's plan of care. In New Mexico this is licensed by the New Mexico Health Care Authority (HCA) — which took over health-facility licensing from the Department of Health on July 1, 2024 — under 8.370.22 NMAC. Unlike a non-medical personal-care agency, a home health agency needs a registered-nurse Director of Nursing, must pass a survey, and — to bill Medicare (its biggest payer) — must be Medicare-certified. This guide walks the whole path, broken down so a first-timer can follow it.

Skilled home health is a bigger build than non-medical home care — plan accordingly. A home health agency is a clinical operation: it needs an RN Director of Nursing, physician-ordered plans of care, a survey before licensure, and (for Medicare) certification to the federal Conditions of Participation. That's more infrastructure than a non-medical personal-care agency — but it opens Medicare, which non-medical agencies can't bill. If you mainly want to provide non-medical personal care, that's a different (lighter) path; this guide is the skilled home health agency.

The good news: no Certificate of Need. Some states cap new home health agencies with a Certificate of Need. New Mexico does NOT have a CON law, so you don't need state permission to enter the market — you need the license and (for Medicare) certification. Confirm current CON status, but New Mexico is a long-standing non-CON state.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with the NM Health Care Authority, CMS, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A licensed, Medicare-certified home health agency (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance (Part 3)
3	Build your clinical leadership	An RN Director of Nursing + administrator (Part 4)
4	Get the HCA license	Application, policies, the initial survey (Part 4)
5	Staff & train	Nurses, therapists, 75-hour home health aides (Part 5)
6	Get Medicare-certified	CMS-855A + accreditation or state survey (Part 6)
7	Enroll for Medicaid	Turquoise Care provider enrollment + MCOs (Part 8)
8	Add private pay & VA	Private-duty, LTC insurance, VA (Part 8)
9	Get your first patients	Physicians, discharge planners, the ADRC (Part 10)

10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)
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Rule of thumb. Your RN Director of Nursing is the long pole — you can't be licensed or certified without one. Line that up first, build your clinical policies to the federal Conditions of Participation, then license with HCA and pursue Medicare certification (accreditation is the faster route).

PART 1

What This Business Is

In short: A licensed home health agency — skilled nursing, therapy, and home health aide services in patients' homes, under a physician's plan of care. Paid mainly by Medicare, plus Medicaid and private pay.

New Mexico (8.370.22 NMAC) requires a home health agency to provide at least one medically directed service — skilled nursing, physical therapy, occupational therapy, speech therapy, or home health aide services — supervised by a licensed professional. It's a clinical, physician-ordered service for patients who need part-time or intermittent skilled care at home, often after a hospital stay.

Home health (skilled) vs. non-medical personal care

	Home health agency (this guide)	Non-medical personal care
What it is	Nursing, therapy, aide under physician orders	Help with daily living (no skilled care)
License	HCA home health agency license (survey required)	Delivered as a Medicaid benefit; no facility license
Leadership	RN Director of Nursing required	No RN required
Biggest payer	Medicare (must be certified)	Private pay / Medicaid personal care

Key terms. HCA = NM Health Care Authority (your licensor, since July 2024). 8.370.22 NMAC = the home health agency rule. DON = Director of Nursing (RN). CoPs = the federal Home Health Conditions of Participation (42 CFR Part 484). CMS-855A = the Medicare enrollment application. Turquoise Care = NM Medicaid managed care (since July 2024). Community Benefit = the Medicaid LTSS/personal-care benefit. CCHSP = Caregivers Criminal History Screening Program. EVV = Electronic Visit Verification (AuthentiCare). NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. New Mexico is one of the oldest and most rural states in the country, and its 65-and-over population is growing fast across counties with few facilities — which means strong, steady demand for skilled care delivered where people already live: at home. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: *HCA licenses and surveys you, CMS certifies you for Medicare, the Turquoise Care MCOs pay you for Medicaid, and the CCHSP and Employee Abuse Registry screen your staff.*

NM Health Care Authority — Program Operations Bureau (your licensor)

The New Mexico Health Care Authority (formerly the Department of Health for this function) licenses home health agencies under 8.370.22 NMAC, conducts the initial and unannounced surveys, and administers Medicaid. HCA Health Facility Licensing & Certification; hca.nm.gov.

CMS / your Medicare Administrative Contractor

To bill Medicare — the largest home-health payer — you enroll with CMS (Form 855A through your Medicare Administrative Contractor, Novitas) and demonstrate compliance with the federal Conditions of Participation, either through accreditation (ACHC, CHAP, or The Joint Commission) or a state survey.

Turquoise Care MCOs (Medicaid)

New Mexico Medicaid is Turquoise Care (since July 1, 2024), delivered through four managed-care organizations: Blue Cross Blue Shield of New Mexico, Molina Healthcare, Presbyterian Health Plan, and UnitedHealthcare Community Plan. You enroll as a NM Medicaid provider and contract with each MCO.

Background screening (CCHSP & the Employee Abuse Registry)

Every direct-care worker must pass the Caregivers Criminal History Screening (fingerprint-based FBI + NM DPS, via IdentoGO — 8.370.5 NMAC), applied for within 20 days of hire, and you must check the Employee Abuse Registry (8.370.8 NMAC) BEFORE employing anyone. CCHSP: (505) 423-4637.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In New Mexico you form the LLC through the Secretary of State’s online Business Services portal (505-827-3600), then register for the state’s gross-receipts (CRS) tax with NM Taxation & Revenue. You’ll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization.** Through the NM Secretary of State at sos.nm.gov (Business Services; 505-827-3600) — the filing fee is about \$50; confirm the current amount.
- 2. Name a registered agent and adopt an Operating Agreement.** The agent (which can be you) receives legal mail at a New Mexico physical address; the Operating Agreement sets ownership and management.
- 3. Register for NM gross-receipts (CRS) tax.** With NM Taxation & Revenue, so you can report the state’s gross-receipts tax and set up to run payroll.
- 4. Note: New Mexico LLCs file NO annual report.** Unlike most states, NM does not require an annual LLC report or franchise fee — one less recurring deadline to track.

No newspaper publication in New Mexico. *Unlike some states, New Mexico has no LLC publication requirement — forming the entity is a straightforward filing plus the CRS tax registration.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business’s federal tax number — think of it as a Social Security number for the company. It’s free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here’s exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party’s legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can’t be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You’ll use it on your Medicare enrollment (Form 855A) and to enroll with NM Medicaid and bill the Turquoise Care MCOs. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home health agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of New Mexico branches — Wells Fargo, Bank of America, or U.S. Bank, or a New Mexico option like Nusenda Credit Union or a local community bank. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home health agency sends nurses, therapists, and aides into patients' homes under physician orders, so you need coverage built for clinical risk — professional (malpractice) liability is foundational, and New Mexico requires workers' compensation for employees:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a home, plus clinical malpractice claims about the CARE (a nurse's or therapist's error)	Yes — foundational
Abuse & Molestation	Allegations of abuse by staff — general liability EXCLUDES this	Yes — high exposure
Non-Owned & Hired Auto	Your nurses, therapists, and aides driving their own cars to patients	Yes — critical for home health
Workers' Compensation	Staff injuries (lifting and transferring patients)	Required in New Mexico for employees
Cyber Liability	A breach of protected health information (OASIS and clinical records)	Strongly recommended

Real places to get it

Buy through an independent agent or broker who writes skilled home-health programs. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, and Markel, plus home-health specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local New Mexico agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care). Payers and accreditors will require proof of professional liability, so confirm the carrier writes skilled home health in New Mexico before you rely on a quote.

What to ask the insurance agent

“Do you write skilled home health agencies — which carriers, and what's the minimum premium?”

“Is professional liability (clinical malpractice) included, and is abuse & molestation included or a sub-limit?”

“Do you include non-owned & hired auto (my nurses and therapists drive their own cars)?”

“What coverage limits do Medicare accreditation, the Turquoise Care MCOs, and the VA require?”

PART 4

Get Your HCA License — Step by Step

In short: Build your clinical leadership and policies, apply to HCA, and pass the initial survey. A home health license requires an RN Director of Nursing and a survey before it's issued.

4.1 Your clinical leadership (required)

- **RN Director of Nursing / supervising nurse:** medically directed services must be supervised by a licensed professional, and home health aides must be trained by or under an RN with at least two years of nursing experience. This RN is essential — line them up first.
- **Administrator:** a physician, an RN, or a person with a qualifying degree plus at least one year of supervisory home-health experience.
- **Documented RN supervisory visits:** at least every 60 days per residence (every 14 days for patients receiving skilled care under Medicare).

4.2 The application & survey

1. **Build your clinical policies.** To 8.370.22 NMAC and the federal Conditions of Participation (42 CFR Part 484) — plan of care, skilled services, aide training/supervision, QAPI, infection control, patient rights, emergency preparedness.
2. **Apply to HCA.** Submit your application and the required fee to HCA's Program Operations Bureau. (The rule references a fee but doesn't state the amount — confirm the current license/application fee with HCA.)
3. **Pass the initial survey.** An initial on-site survey is required before HCA issues the annual license; expect unannounced surveys thereafter.
4. **Receive your license.** Then keep it current and survey-ready.

4.3 Timeline

Between hiring your DON, writing clinical policies, and scheduling the survey, plan for several months. Run your Medicare certification (Part 6) and Medicaid enrollment in parallel where you can.

PART 5

Your Clinical Team & Aides — by Position

In short: A home health agency runs on licensed clinicians and trained aides. Here's the team and the training/screening rules.

5.1 Nurses & therapists

You'll employ or contract RNs and LPNs (LPNs work under an RN with the required experience) and physical, occupational, and speech therapists as your service mix requires — all licensed in New Mexico and meeting the federal personnel qualifications (42 CFR 484.115). Your DON oversees the clinical program.

5.2 Home health aides — 75-hour training (42 CFR 484.80)

- **Training:** at least 75 hours total, including at least 16 classroom hours before at least 16 hours of supervised practical training.
- **Competency:** a written and performance competency evaluation before the aide furnishes services.
- **In-service:** at least 12 hours per 12 months.
- **Supervision:** RN supervisory visits (every 14 days when the patient is receiving skilled care; every 60 days for aide-only).

5.3 Background screening — the system you must run

1. **Run the Caregivers Criminal History Screening (8.370.5 NMAC).** A fingerprint-based FBI + NM Department of Public Safety check via IdentoGO/IDEMIA (fee commonly cited around \$88 — confirm), applied for within 20 calendar days of first employment. CCHSP: (505) 423-4637.
2. **Check the Employee Abuse Registry (8.370.8 NMAC) BEFORE hiring.** You may not employ anyone listed; keep documentation — civil penalties up to \$5,000 per instance.
3. **Honor the disqualifying convictions.** 8.370.5.11 lists them (homicide, trafficking, sexual offenses, abuse/neglect, enumerated property felonies, and attempts/conspiracies).
4. **Do TB screening.** All staff, contractors, or volunteers with patient contact must have a TB test per the NM Infectious Disease Bureau requirements.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

Get Medicare-Certified — Step by Step

In short: Medicare is the biggest home-health payer, and you can't bill it without certification. Enroll with CMS and demonstrate compliance with the Conditions of Participation — accreditation is usually the faster route.

6.1 The certification path

1. **Get your NPI and your HCA license first.**
2. **Enroll with CMS (Form 855A).** Submit the 855A to your Medicare Administrative Contractor (Novitas) to enroll the agency in Medicare.
3. **Demonstrate compliance with the Conditions of Participation (42 CFR Part 484).** Either (a) get accredited by a CMS-approved accreditor — ACHC, CHAP, or The Joint Commission — which grants “deemed status,” or (b) take a state survey conducted by HCA on CMS's behalf. Accreditation is usually faster and predictable.
4. **Serve the required patients for survey.** A home health agency generally must be operating and serving patients to complete its initial certification survey — plan a private-pay/self-pay start to build census.

Accreditation does double duty. ACHC, CHAP, or Joint Commission accreditation both satisfies Medicare's survey requirement (deemed status) and is a quality credential payers and referral sources value. Budget roughly \$7,000–\$14,500 over a 3-year cycle plus staff time, and build your policies to the accreditor's standards from the start.

6.2 Medicaid & EVV setup

Separately, enroll as a New Mexico Medicaid provider (through the state Medicaid portal) and contract with the Turquoise Care MCOs (Part 8). For Medicaid personal-care and home-based services, New Mexico uses AuthentiCare for Electronic Visit Verification — confirm the current operator and set it up before billing those services.

6.3 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Patient's Journey — Referral to Discharge

In short: *A patient is referred with physician orders, your nurse assesses and builds the plan of care, your clinicians and aides deliver the visits, you supervise and document, and you discharge when goals are met.*

Stage 1 — Referral & physician orders

A referral comes from a physician, a hospital discharge planner, or a skilled nursing facility (Part 10). Skilled home health requires physician (or allowed practitioner) orders and a face-to-face encounter, for a patient who is homebound and needs part-time/intermittent skilled care.

What you confirm on intake:

- Who the ordering physician is and that the face-to-face encounter is documented; that the patient is homebound and needs a skilled service; the payer (Medicare, Turquoise Care MCO, private, or VA) and any authorization; and that the ordered services are within your staffed scope.

Stage 2 — Assessment & the plan of care

Your RN completes the start-of-care assessment (the OASIS data set for Medicare) and, with the physician, establishes the plan of care — the ordered skilled services, therapies, aide visits, frequency, and goals.

What the RN assesses in the home:

- Clinical status and medications; fall and safety hazards; the caregiver and family supports at home; equipment needs; and the specific measurable goals the ordered visits will work toward — all of which the physician must sign off on.

Stage 3 — Start of care

The patient signs consents and receives a rights orientation. Your clinicians begin the ordered visits; any home health aide works under the RN's supervision and the plan of care.

Stage 4 — Delivering the visits

Nurses, therapists, and aides deliver the ordered visits and document each one (with EVV where Medicaid requires it). Coordinate care with the physician and adjust as the patient progresses.

Stage 5 — Supervision, QAPI & recertification

Your DON runs the clinical program and Quality Assessment and Performance Improvement (QAPI); RN supervisory visits occur on the required schedule; and Medicare episodes are recertified with updated orders as needed.

Stage 6 — Reporting & compliance

Submit OASIS data, maintain infection control and emergency-preparedness compliance, and report/handle incidents. Everything is documented for survey and payer audit.

Stage 7 — Discharge

When the patient meets their goals or no longer needs skilled care, discharge per the plan of care with physician sign-off, coordinate any ongoing non-skilled needs (e.g., a personal-care referral), and document the outcome.

PART 8

How You Get Paid — Medicare, Medicaid (Turquoise Care), Private Pay & VA

In short: Medicare is the primary payer for skilled home health once you're certified; Medicaid (Turquoise Care) and private pay/LTC insurance round it out, and the VA is a real channel.

Your revenue streams:

Source	What it is	Key fact
Medicare	Skilled home health for homebound patients	The biggest payer — requires Medicare certification
Medicaid — Turquoise Care	Home health + Community Benefit personal care	Through the 4 MCOs; requires enrollment, contracts & (for personal care) EVV
Private pay / LTC insurance	Self-pay skilled or private-duty; LTC policies	Private-duty nursing and LTC-insurance-covered care
Veterans (VA)	VA Homemaker/HHA + skilled care	Serve veterans via the Community Care Network (Region 4, TriWest)

8.1 Medicare — your primary payer

Once you're Medicare-certified, Medicare covers part-time/intermittent skilled nursing, therapy, and home health aide services (aide only if also getting skilled care) for homebound patients under a physician's plan of care, at \$0 to the patient for covered services. Medicare pays under its home-health payment model; build your clinical and billing operation to the Conditions of Participation.

8.2 Medicaid — Turquoise Care

New Mexico Medicaid (Turquoise Care) covers home health and, through the Community Benefit, personal care for members meeting a nursing-facility level of care — in an Agency-Based Community Benefit model (your agency employs the caregivers) and a Self-Directed Community Benefit model (the member self-directs). Enroll as a NM Medicaid provider and contract with each MCO (BCBS NM, Molina, Presbyterian, UnitedHealthcare); Medicaid enrollment alone doesn't put you in-network. Pull current rates from the HCA/Medicaid fee schedules.

8.3 Private pay, LTC insurance & VA

- **Private pay & LTC insurance:** private-duty skilled care and personal care not covered by a payer; New Mexico non-medical care runs about \$30/hour (2025 data — verify), and private-duty nursing far higher. Long-term-care insurance reimburses covered home care once benefit triggers are met.
- **Veterans:** the VA's Homemaker/Home Health Aide and skilled community-care programs — contract through the VA Community Care Network. New Mexico is in Region 4, administered by TriWest. Aid & Attendance can help a veteran privately fund care (verify amounts on VA.gov).

8.4 Enrolling with Turquoise Care — the Medicaid step-by-step

Getting paid by New Mexico Medicaid is a two-part process that trips up new owners: enrolling as a state Medicaid provider is NOT the same as being in a health plan's network. You have to do both — enroll with the state, then contract with each of the four Turquoise Care managed-care organizations separately. Here is the order that works:

1. **Get your NPI and HCA license first.** Turquoise Care enrollment requires your Type 2 NPI and your active HCA home health agency license — have both in hand before you start.
2. **Enroll as a NM Medicaid provider.** Apply through the HCA/Medicaid provider enrollment portal (hca.nm.gov) to get your New Mexico Medicaid provider ID. This registers you with the state — it does NOT put you in any plan's network.
3. **Contract and credential with all four MCOs.** Blue Cross Blue Shield of New Mexico, Molina Healthcare, Presbyterian Health Plan, and UnitedHealthcare Community Plan each run their own contracting and credentialing — apply to all four so members on any plan can choose you.

4. Set up AuthentiCare EVV before your first personal-care visit. For Community Benefit personal-care and home-based services, New Mexico verifies visits through AuthentiCare — onboard, load your workers and authorizations, and confirm the current operator before you bill.

Enrollment is not the same as being in-network. A common and costly mistake: an agency enrolls with NM Medicaid, starts serving members, then finds its claims deny because it never signed a contract with that member's MCO. Enroll with the state AND contract with all four Turquoise Care plans — and confirm each plan's authorization and timely-filing rules — before you count on Medicaid revenue.

PART 9

What It Costs & How Long It Takes

In short: A home health agency is a bigger build than non-medical care — budget clinical leadership, accreditation, the survey, and working capital for Medicare's payment cycle.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
HCA license	Initial home health agency license + survey	Confirm fee with HCA
RN Director of Nursing	Required clinical leadership	Ongoing salary
Clinical staff	Nurses, therapists, aides	Ongoing payroll
Accreditation (for Medicare)	ACHC / CHAP / Joint Commission (deemed status)	~\$7,000–\$14,500 / 3-yr cycle
Business setup	SOS LLC ~\$50 (no annual report), EIN free, NPI free, legal	~\$50 + legal
Insurance	Professional + abuse + non-owned auto + workers' comp	Get quotes — annual
Software	EHR/OASIS + scheduling/billing/payroll	Monthly SaaS
Working capital	Bridge Medicare's initial payment cycle	Significant — plan ahead

Plan for the Medicare cash-flow cycle. New home health agencies often wait months to see steady Medicare payments while they build census and complete certification. Capitalize the business accordingly, and consider a private-pay/self-pay start to generate cash while certification proceeds.

9.2 Timeline

First	Hire your DON	Your RN Director of Nursing gates everything
Early	Business & policies	Form the LLC, write clinical policies to the CoPs
Months	HCA license → survey	Apply → initial on-site survey → license
Months	Medicare certification	855A + accreditation (deemed status) or state survey
Parallel	Medicaid & VA	Turquoise Care enrollment + MCO contracts; TriWest CCN
Ongoing	Referrals & census	Physicians, discharge planners, the ADRC

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$30/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Working capital for the Medicare cycle:** you pay nurses, therapists, and aides every week or two, but Medicare and the Turquoise Care MCOs pay you weeks to months after you bill — and a new agency waits even longer while census and certification ramp. Keep a real cash reserve, or start on private pay while your Medicare and Medicaid lines mature.
- **Unbillable clinical overhead:** your DON's supervisory visits, OASIS review, QAPI meetings, on-call coverage, and care coordination with physicians are all required and none of it is billable — but all of it is real payroll.
- **Turnover churn:** every nurse, therapist, or aide who quits costs you re-recruiting, re-screening (the CCHSP fingerprint check and Employee Abuse Registry check), and re-orienting. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Denied and delayed claims:** a Medicaid claim that doesn't match a verified AuthentiCare visit, or that misses an MCO's timely-filing window, simply doesn't get paid. Reconcile every remittance and chase exceptions weekly.
- **Accreditation renewal:** if you take the accreditation route for Medicare, it isn't one-and-done — it runs on a three-year cycle with ongoing fees and a re-survey. Calendar it so it never lapses.

PART 10

Getting Your First Patients

In short: *Skilled home health runs on physician orders — so physicians and discharge planners are your pipeline, along with SNFs, the aging network, and senior communities. Build these before you open.*

10.1 The referral sources

- **Physicians & practices:** skilled home health requires physician orders and a face-to-face encounter — build relationships with primary-care, geriatric, cardiology, and post-surgical practices.
- **Hospital discharge planners & case managers:** the highest-volume source for post-acute skilled care — make it easy to refer and start quickly.
- **Skilled nursing facilities & rehab:** step-down referrals for patients going home with continuing skilled needs.
- **The aging network:** New Mexico's Area Agencies on Aging and the Aging & Disability Resource Center (ADRC, 1-800-432-2080) route older adults to services — get listed and known.
- **Senior living communities & senior centers:** referral partners for residents needing skilled care at home.
- **NM 2-1-1:** list your services (dial 211) — a statewide referral channel.

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, certification & staff

Who	Contact	What to verify / ask
NM Health Care Authority — licensing	hca.nm.gov (Health Facility Licensing)	The home health license (8.370.22 NMAC), the fee, and the initial survey
CMS / Novitas (Medicare)	cms.gov · novitas-solutions.com	The 855A enrollment; the Conditions of Participation; the certification survey
Accreditor — ACHC / CHAP / TJC	achc.org · chapinc.org · jointcommission.org	Deemed status for Medicare; cost and timeline
CCHSP (background screening)	(505) 423-4637 · CCHSPrequests@hca.nm.gov	The fingerprint screening + the Employee Abuse Registry check

Business, insurance & billing IDs

Who	Contact	What to verify / ask
NM Secretary of State	505-827-3600 · sos.nm.gov	Forming your LLC (~\$50); no annual report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	nppe.cms.hhs.gov	Your Type 2 NPI (for Medicare/Medicaid billing)
Insurance agent (home-health program)	trustedchoice.com ; carriers in Part 3.3	Professional + abuse + non-owned auto + workers' comp

Funding & referrals

Who	Contact	What to verify / ask
Medicare	1-800-MEDICARE · medicare.gov	Covered home-health services and the payment model
NM Medicaid / Turquoise Care	hca.nm.gov/turquoise-care	Provider enrollment; the 4 MCOs; the Community Benefit; the rates
NM Medicaid — EVV / AuthentiCare	hca.nm.gov	EVV setup for personal-care/home-based services
VA Community Care / TriWest (Region 4)	vacn.triwest.com	The VA home-care programs; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Aging & Disability Resource Center (ADRC)	1-800-432-2080	The statewide no-wrong-door aging & disability line
Area Agencies on Aging	via NM Aging & Long-Term Services	Local information & referral for older adults
NM 2-1-1	dial 211	Free statewide referral line — list your services
Physicians & discharge planners	your local practices & hospitals	The pipeline for physician-ordered skilled care

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening a New Mexico home health agency is a substantial, clinical build — an RN Director of Nursing, a survey, and Medicare certification — but it opens the largest home-care payer (Medicare) and a real Medicaid and VA market. Take it one rung at a time: hire your DON, form the business, build clinical policies to the Conditions of Participation, get the HCA license, get Medicare-certified (accreditation is the faster route), enroll for Turquoise Care, and build physician and discharge-planner relationships. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in 8.370.22 / 8.370.5 / 8.370.8 NMAC, the federal Conditions of Participation (42 CFR Part 484), and current Turquoise Care information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with the NM Health Care Authority, CMS, and qualified counsel before you rely on it. Confirm the current license fee, the Medicaid rates, and your EVV setup before you rely on them.